



PSYC1003

Dissociation & Dissociative Identity Disorder

Part 2: Is Dissociative Identity Disorder real?

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Is DID real? Evidence

- Different allergic responses (Braun, 1983)
- Different responses to the same medication (Kluft, 1984)
- Different handedness (Kluft, 1996)
- **Hand-writing differences b/w alters**
- Lewis, et al (1997). Objective documentation of child abuse & dissociation in 12 murderers with dissociative identity disorder. *American Journal of Psychiatry*, 154, 1703-1710
- **Optical variability amongst alters**
- Miller et al (1991). Optical differences in multiple personality disorder: A second look. *The Journal of Nervous & Mental Disease*, 179, 132-135

*I don't want that shit done for the people
that wronged me!!!*

*When I was in a jail cell / sitting in a jail
cell for a crime someone else had done I guess that*

*happened. Just as the names that I use actually
belong to people I know...*

By
M L

By Johnny7

Franklin

DID evidence: *psychogenic blindness*

Totally blind woman diagnosed with DID

- Blindness initially attributed to craniocerebral trauma but *change after therapy*
- Absent visual evoked potentials (VEP) in the blind identity versus normal VEP in the seeing identity
- Strasburger, H., & Waldvogel, B. (2015). Sight & blindness in the same person: Gating in the visual system. *PsyCh Journal*, 4, 178-185
- Bhuvaneswar, C., & Spiegel, D. (2013). An eye for an I: A 35-year-old woman with fluctuating oculomotor deficits & dissociative identity disorder. *International Journal of Clinical & Experimental Hypnosis*, 61, 351–370

DID: brain imaging evidence

Functional brain imaging (PET & fMRI) differences b/w DID & controls (Reinders et al, 2003, 2006; Sar et al, 2007; Schlumpf et al, 2013)

- Differences not found when *simulating identities* (Reinders et al, 2012)

Reinders, AATS, et al (2003) One brain, two selves. *NeuroImage*, 20, 2119–2125

Reinders, AATS, et al (2006) Psychobiological characteristics of dissociative identity disorder: A symptom provocation study. *Biological Psychiatry*, 60, 730–740

Sar, V. et al (2007). Frontal & occipital perfusion changes in dissociative identity disorder. *Psychiatry Research: Neuroimaging*, 156, 217-223.

Schlumpf, Y. et al (2013). Dissociative part-dependent biopsychosocial reactions to backward masked angry & neutral faces: An fMRI study of dissociative identity disorder. *NeuroImage: Clinical*, 3, 54-64

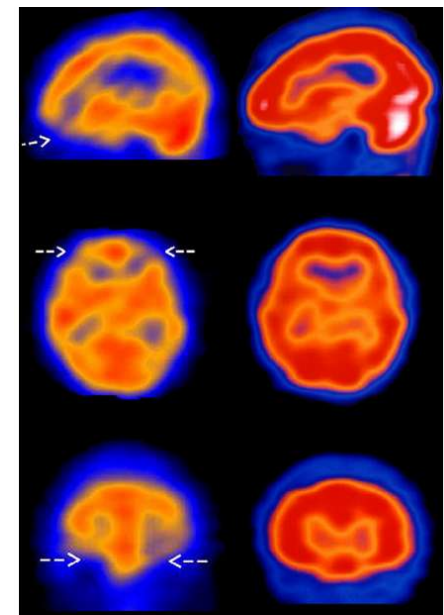
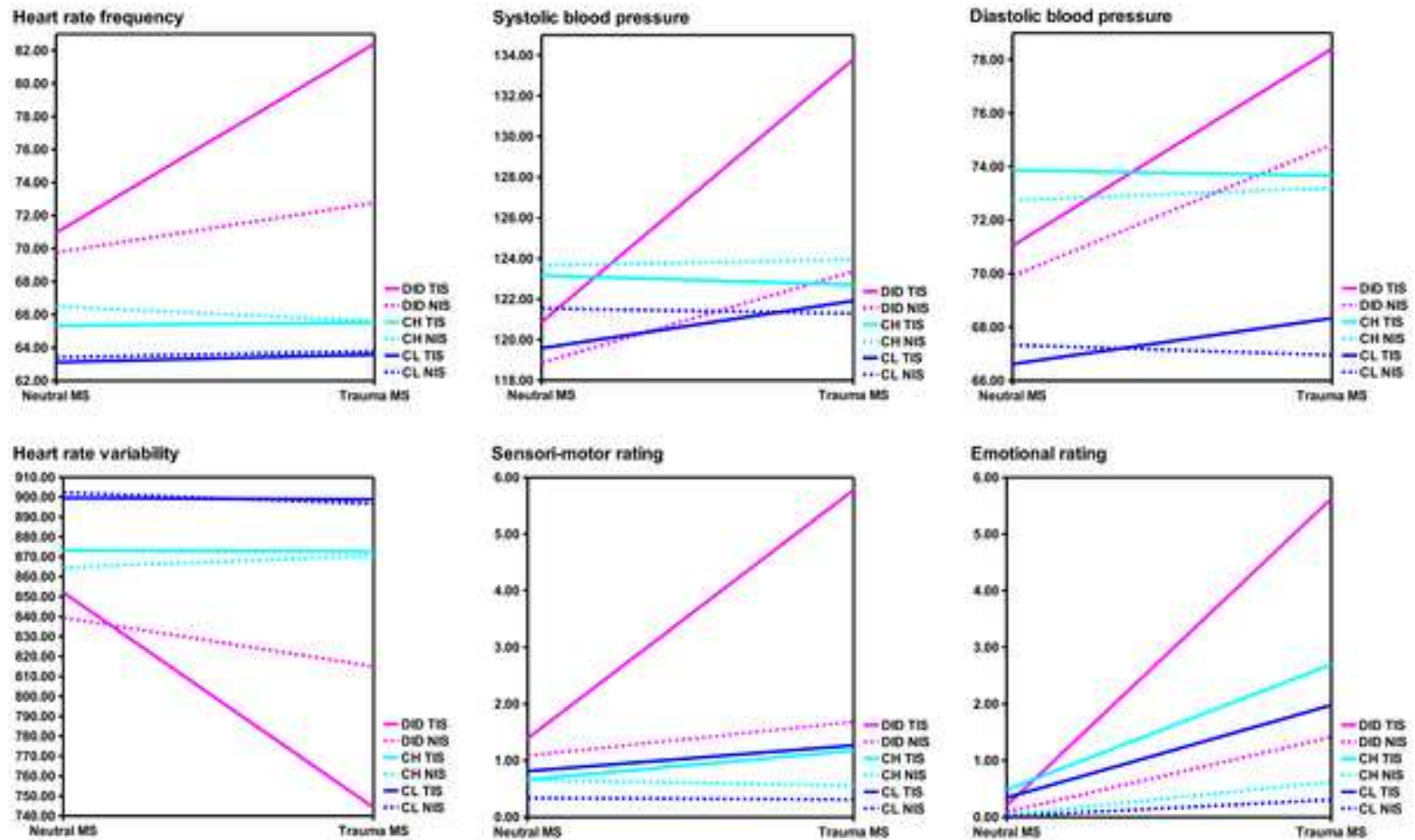


Fig. 2. Visually observed orbitofrontal perfusion defects in a patient with dissociative identity disorder (left) compared with the cerebral perfusion in a healthy volunteer (right).



Reinders AATS, et al (2012) Fact or factitious? A psychobiological study of authentic & simulated dissociative identity states. *PLoS One*, 7, e39279



DID & neuroanatomy

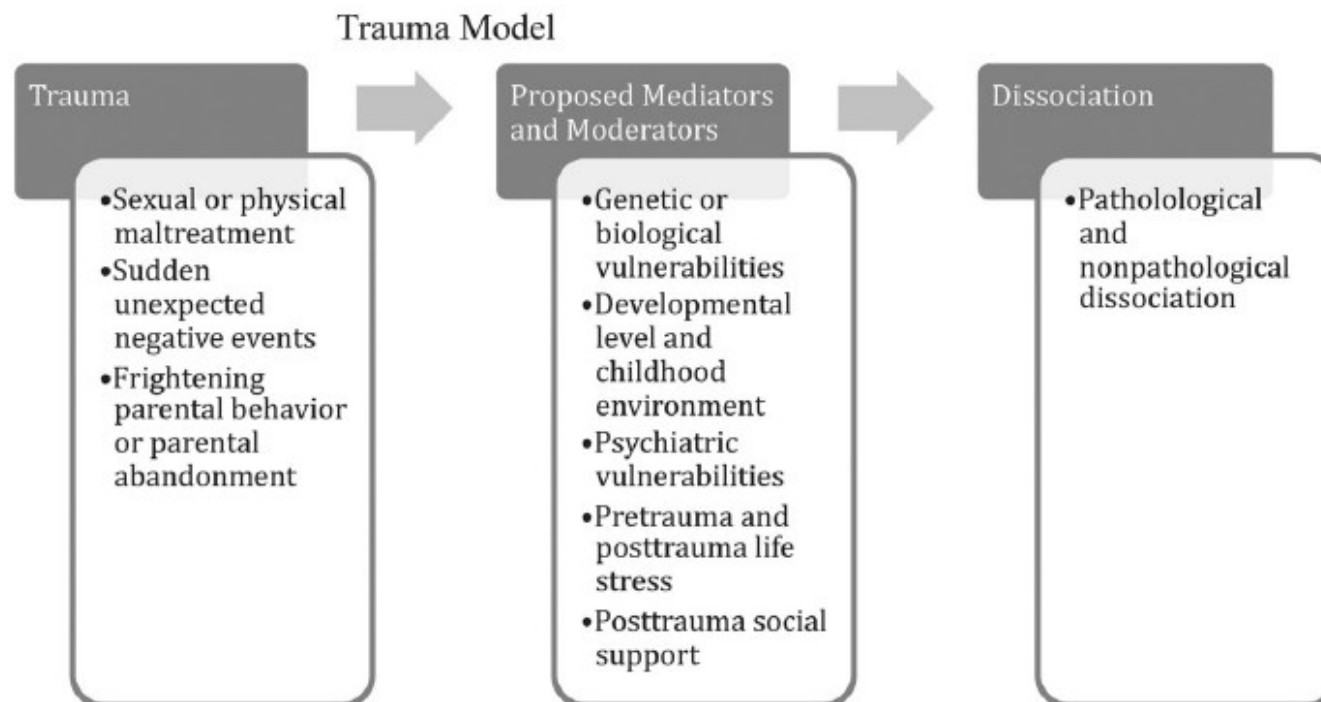
Blihar et al. (2020): systematic review (10 MRI studies); DID vs healthy controls

- **DID:** ↓ hippocampal volumes (*memory consolidation*); ↓ amygdala (*defenses*); ↓ parietal structures (*personal awareness*)
- *Brain markers can potentially diagnose DID(?)*
- **Reinders et al. (2019): MRI study** (32 DID vs 43 healthy controls)
- ML pattern classifiers 72-74% accurate
- Blihar, et al. (2020). A systematic review of the neuroanatomy of dissociative identity disorder. *European Journal of Trauma & Dissociation*, 4, 100148
- Reinders et al. (2019). Aiding the diagnosis of dissociative identity disorder: pattern recognition study of brain biomarkers. *The British Journal of Psychiatry*, 215, 536-544



a) Theories of DID: trauma model

- *Trauma leads to DID via various biopsychosocial mediators & moderators*



Trauma & DSM 5

Traumatisation: In the context of family & attachment pathology, early life trauma (e.g., neglect & physical, sexual, & emotional abuse, usually before ages 5–6 years)

“Individuals with the disorder typically report multiple types of interpersonal maltreatment during childhood & adulthood”

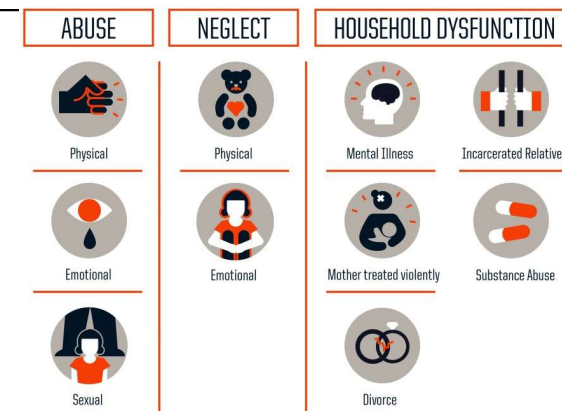
“They are *intentional* acts by other human beings (*interpersonal*) that are *inescapable* & create a sense of fundamental *insecurity*”
(Ford, 2017)



Complex trauma

Also known as:

- Attachment trauma
- Type II trauma
- Early relational trauma
- Developmental trauma



“CT involves cumulative, polyvictimization perpetrated within the caregiving system during childhood by adults who are expected to provide security, protection, & stability. For these children, CT is an ongoing & stable condition rather than a time-limited event”

(Lawson & Akay-Sullivan, 2020)

DID & complex trauma



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Majority of cases report incest or brutal sexual abuse often by a psychotic adult

- eg. physical damage to the genitals (eg. with icepick), locking children in closet for days, torture of children's pets
- ***DID as an initially adaptive defensive response:*** mental splitting/dissociation
- Number of alters associated with severity/duration of abuse (Goodwin & Sachs, 1996)



DID & complex trauma

Dalenberg et al (2012): evidence supports view that complex trauma causes DID

“Every study that has systematically examined aetiology has found that antecedent severe, chronic childhood trauma is present in the histories of almost all individuals with DID”

(Dorahy et al, 2014)

- **Neurobiological evidence suggests DID is a severe form of PTSD** (Reinders & Veltman, 2021)

Dorahy, et al (2014). Dissociative identity disorder: An empirical overview. *Australian & New Zealand Journal of Psychiatry*, 48, 402-417

Reinders & Veltman (2021). Dissociative identity disorder: out of the shadows at last? *The British Journal of Psychiatry*, 219, 413-414



Structural theory of Dissociation

(van der Hart et al., 2006)

- Based on Janet's theory & later study of acutely traumatised World War I combat soldiers (Myers, 1940)

Personality system splits into:

- **Trauma-avoidant *Apparently Normal Part (ANP)*** or ANPs: Daily-life action system(s)
- **Trauma-fixated *Emotional Part (EP)*** or EPs: Fight-defense action system(s)
- “Each part includes at least a rudimentary sense of an experiencing “I”” (Steele et al., 2009)

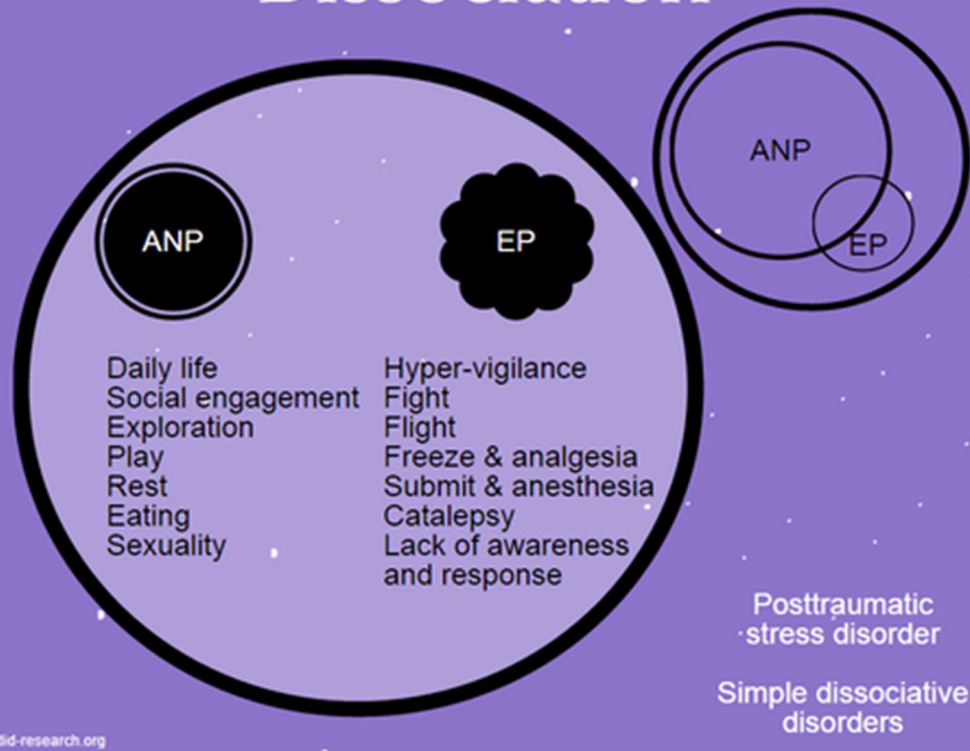
Structural theory of Dissociation

(van der Hart et al., 2006)

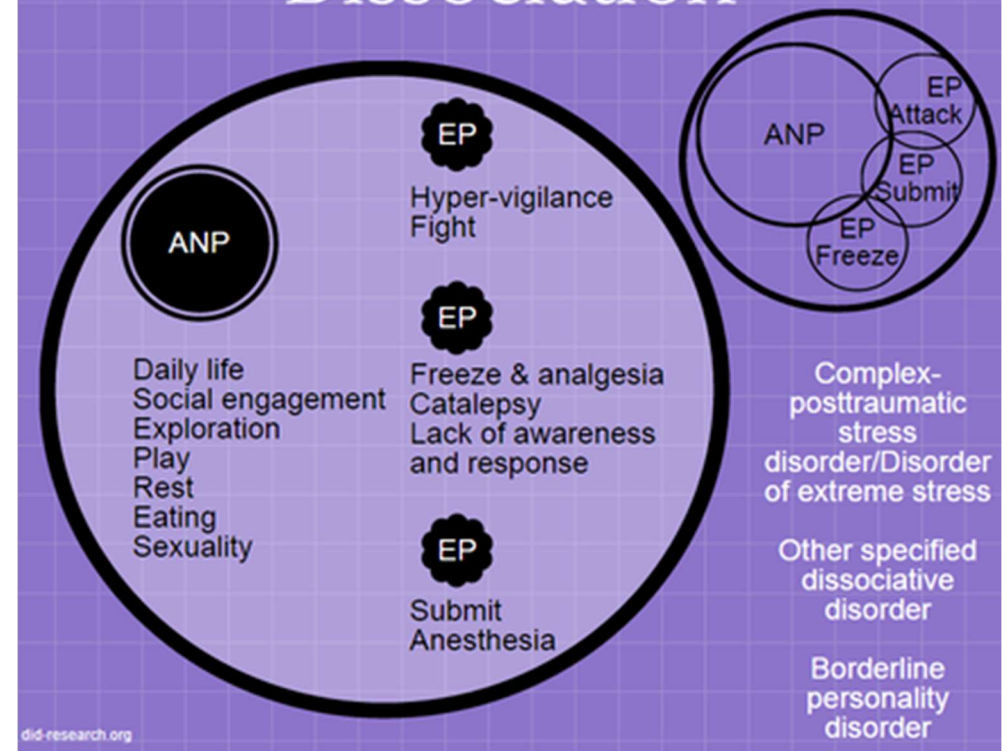


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Primary Structural Dissociation



Secondary Structural Dissociation

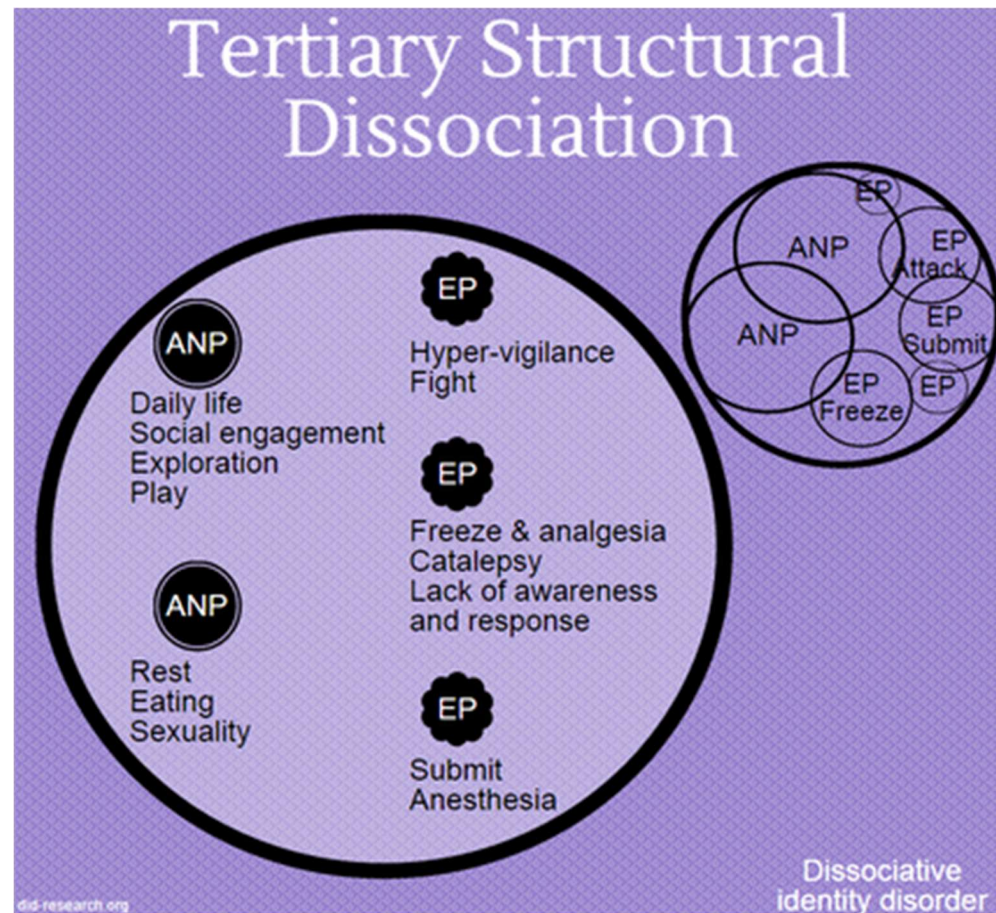


Structural theory of Dissociation

(van der Hart et al., 2006)



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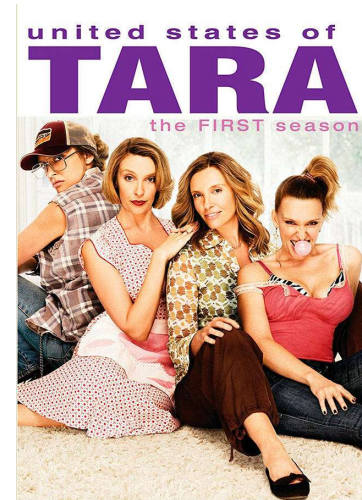
b) Theories of DID: sociocognitive/fantasy model

Importance of social learning & expectancies

- *Trauma can be present but unnecessary*
- DID a disorder of self-understanding

DID results from:

- **iatrogenic factors:** inadvertent therapist cues (eg. suggestive questioning about alters, hypnosis for memory recovery)
- **Media influences** (eg. television & film portrayals of DID)
- **Sociocultural expectations** regarding the presumed clinical features of DID





DID occurs “when people with coexisting or ambiguous psychological symptoms are exposed to suggestive procedures (e.g., repeated questioning about memories & personality “parts,” leading questions, hypnosis, ... media influences (eg, film & television), & broader sociocultural expectations (e.g., “dissociation is associated with abuse,” people possess “multiple personalities”) regarding the presumed clinical features of DID”
(Lynn et al, 2012)

Theories of DID: sociocognitive/fantasy model

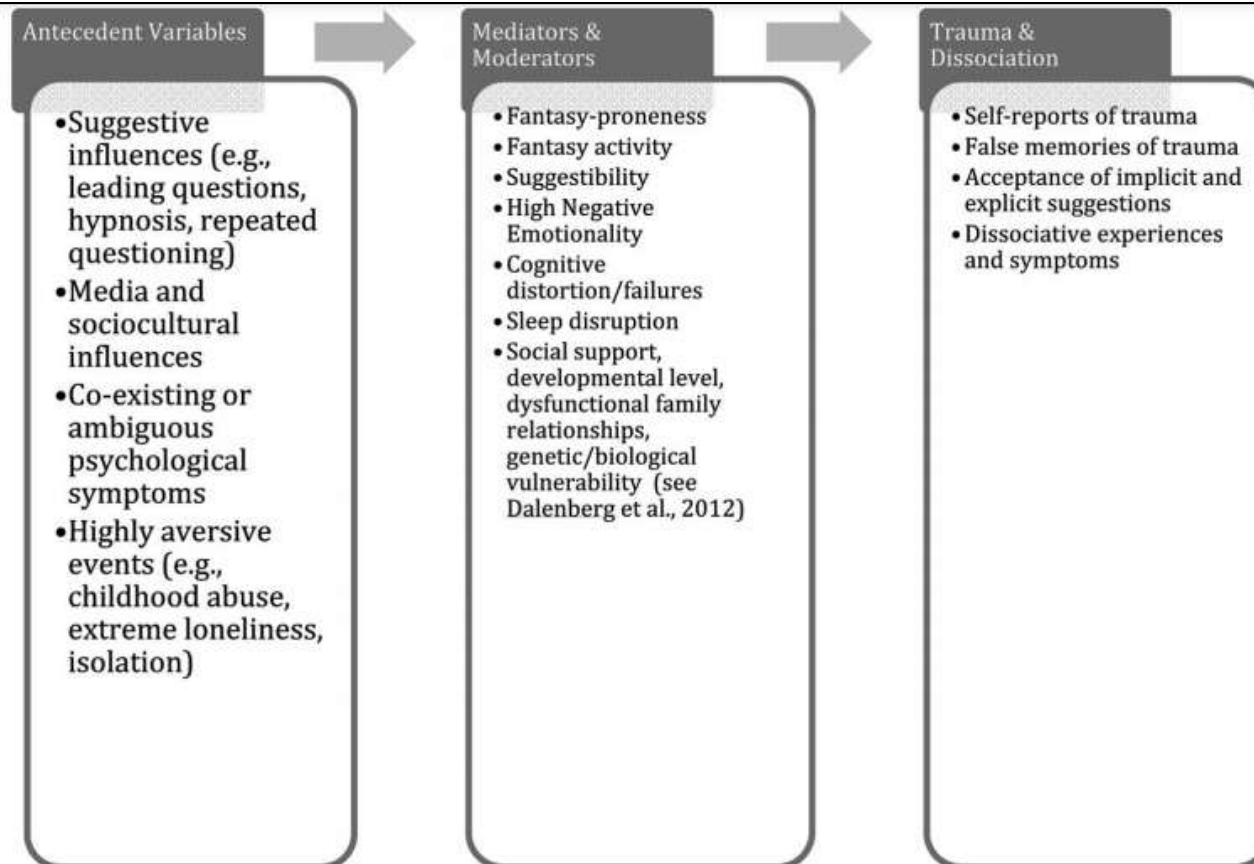


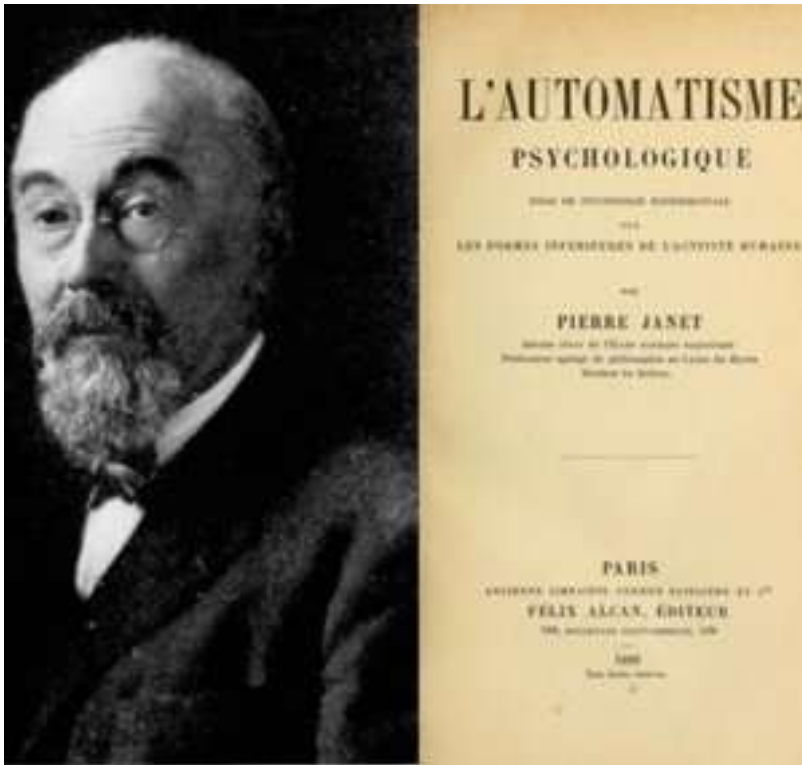
Figure 1. The fantasy model: Key antecedent, mediating, and moderating variables.

Role of trauma?

"We remain open to the possibility that trauma may play a nonspecific causal role in dissociation, largely because the FM [fantasy model] is compatible with the view that a variety of stressors, including not only highly aversive events but also isolation & loneliness ..., can foster the propensity to fantasize, disrupt sleep, & increase vulnerability to suggestive influences..."

(Lynn et al. 2014)

Iatrogenic factors



The following exchange occurred through automatic writing with the patient in hypnosis:

“Do you hear me?”

[response in writing] “No.”

“But in order to reply you must have heard.”

“Yes, surely.”

“Very well, how are you?”

“I don’t know.”

“There must be someone there who hears me.”

“Yes.”

“Who is it?”

“Someone other than Lucie.”

“Ah indeed. Another person.”

“Would you like us to give her a name?”

“No.”

“Yes it would be more convenient.”

“All right. Adrienne.”

“Very well, Adrienne. Do you hear me?”

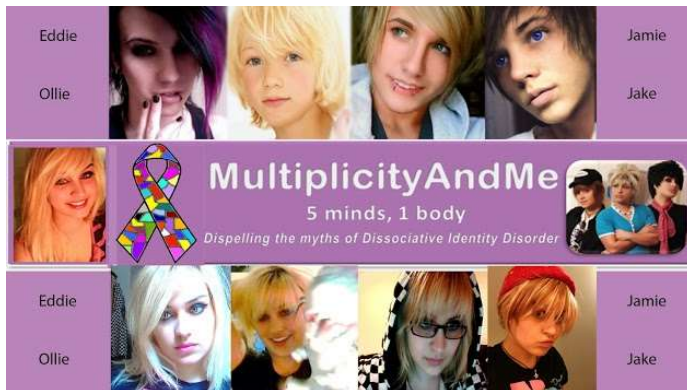
“Yes.”

As late as 1913, Janet remarked, “Without a doubt, I suggested the name of the personage and gave it a lot of individuality, but we saw how much it developed spontaneously . . . this naming of the unconscious personage greatly facilitates the experiences” (124, p 318).

Media influences



- <https://www.youtube.com/watch?v=cjemK803l2M> [MA warning]





“Individuals who feign dissociative identity disorder do not report the subtle symptoms of intrusion characteristic of the disorder; instead they tend to overreport well-publicized symptoms of the disorder, such as dissociative amnesia, while underreporting less-publicized comorbid symptoms, such as depression...

Individuals who feign dissociative identity disorder tend to be relatively undisturbed by or may even seem to enjoy “having” the disorder.

In contrast, individuals with genuine dissociative identity disorder tend to be ashamed of & overwhelmed by their symptoms & to underreport their symptoms or deny their condition...” (DSM-5)



Changes in DID over time

- “Since the 19th century ... the number of personalities per patient has jumped from 2 or 3 to often more than 20 & sometimes into the hundreds” (Spanos, 1994)
- **DSM-III**: Allowed up to 100 alter personalities
- Kluft (1988): 2 cases with more than 4000 personalities or personality fragments

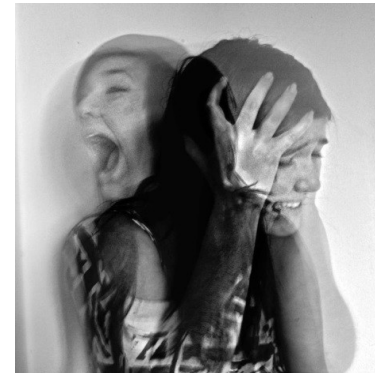




Changes in DID over time

Phenomenology changes

- 19th century cases: *transitional sleep*
- Modern DID: *fast switching*



Changes in alters

- 19th C.: human alters
- 20th C.: *different sex, race, species to host*
- eg. cat, dog, panther, gorilla, lobster, snake
- Demon, angel, God, Hitler, alien
- Robot, tree, river, lake, mountain

Dear Little Ones
Dissociative Identity Disorder
for Young Alters



written by Jade Miller
Illustrated by Germán Zaninetti

Types of Alter Personality (Parts) in Dissociative Identity Disorder

COMMON

Apparently Normal Parts (ANP) / Host
Child alter
Internal Self-helper (ISH)
Introject
Opposite-sex alters
Persecutor
Protector
Sexual Alter
Suicidal Alter/Internal Homicide
Teen alter

LESS COMMON

Animal Alter *
Baby and infant alter
Caretaker/Soother
Demon, Demonic and 'Evil' Alter *
Fragment
Military or Political Alter
Nonhuman Alter
Robot or Machine Alter
Shell
Spirit, Ghost, or Supernatural
. being Alters *
Sub-parts

** These alters see themselves as non-human, and may be unaware they are part of a human mind, they are mentioned in the lengthy DSM-5 DID section.*

No particular types of alter are needed for a DID diagnosis, most people will only recognize a few types. Some people with DID may not recognize any, or may have types not listed. Types of alters depend on what each person needed to survive.

Read more: <http://traumadissociation.com/alters>



Is DID plausible?

“Why did the perhaps half-plausible 19th-century concept so floridly metamorphose into the totally implausible 20th-century concept”

(Piper & Merskey, 2004)

